

العمر(تاريخ الميلاد) : 02-06-1970 المهنة : ربه منزل

رئيس
المجلس الطبي المتخصص

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Name: فاطمه محمود محمد محمد National ID: 27006022301446	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

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03/28/2026

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MDT FORM

Name: Fatma Mohamed Mohamed	MRN: 4889
Age: 56 Y	ID: 27005022301446
Menopausal Status: post menopausal	Phone: 01002979626
NRS SCORE:	Marital Status: married
BMI:	Family History: no
CO Morbidity: free	Reason Of MDT: dp

Clinical Data:

BX:

ER 8/8 **PR** 8/8 **HER** 0 **KI67** 65%

Sonomammography:

17/1/2026: stationary left breast finding birads 6
irregular speculated mass lesions the largest 3.2x2cm
enlarged multiple left axillary lns

Mets Workup:

dynamic mri abdomen&pelvis 2/2026:
few biopolar well defined variable sized focal lesion
the largest 1.3x1.7cm

History:

acase of de novo metastatic hormone positive breast
ldc to liver (single hepatic focal lesion) under femora
since 10/2024

ct abdomen and pelvis 3/8/2025: stationary course of

Recommendations:

MDT Outcome & Plan

for triphasic ct to rule out the nature of the HFL
meanwhile continue on femara
remdt after triphasic

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هاني عزالدين					

07/11/2024, 11:50

code:42 -- Name_A MDT form Hammul -- Name_E MDT form Hammul



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name: **أحمد محمود محمد**

Sex:

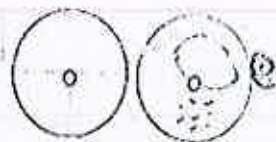
MDT form

3- Medical number:
Female83233105
Old Med:

3225-2024

2-ID number:

Patient Age: 54 Y



Family history

Family history
-ve

Phone

01002879626

BMI

BMI

REASON OF MDT

Breast lump

marital status

Marital status
Married

HRS Score

HRS Score

GROUP

GROUP

Date of MDT

Date of MDT

Co morbidity

Co morbidity
freeMenopausal
statusMenopausal status
Postmenopausal

Initial STAGE

Initial STAGE

07/11/2024

Clinical data

Breast Imaging:

It was locally advanced multicentric malignant mass

Mets Workup:

cap non contrast :suspected It HFL 1.2 x 1 cm
TRIPHASIC LEFT LOBE SEGMENT II LESION

BX IDC BX COMMENT: gii	ER 8/8 ER COMMENT: -ve	PR 8/8 PR COMMENT: -ve	Her 2 Neu Her 2 Neu COMMENT: -ve	SISH SISH COMMENT: -ve	K167 65
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T CT4B	N 2	M 1 SINGLE HFL
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Sonomammography

PATIENT HISTORY

BONE SCAN

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR FIRST LINE AT THEN CONSIDER LT MRM IF RESPONDING

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY Signature	RADIOLOGY	RADIOLOGY Signature
SURGERY	SURGERY Signature	PATHOLOGY	PATHOLOGY Signature
Psychiatric	Psychiatric Signature	Nutrition	Nutrition Signature



Patient Name : لاطمة محمود محمد محمد
Patient No. : 27006022301446
Path No. : 5966/2024
Sex : Female
Age : 54 Y
Referring Doc : nesma fathey

Request No. : 110107698
Request Date : 14/08/2024
Verification Date : 25/08/2024
Financial Category : مبدئية
Department : مبدئية أورام الثدي - سوهاج

Pathology Report

CLINICAL DIAGNOSIS: Left breast mass.

NATURE OF SPECIMEN: TCNB.

GROSS EXAMINATION: The specimen consists of four tissue cores (2cm long) each, grayish in color, totally submitted in one cassette.

MICROSCOPIC PICTURE: Sections were reviewed.

DIAGNOSIS: labeled as left breast mass, TCNB:

-Features of invasive duct carcinoma, NST, grade II.

-Single focus of in-situ component, solid and cribriform pattern was detected.

Signature

Pathologist. Dr: Sarah Rafat

Dr: Christen Nady

N.B : Specimens received at the department of pathology are examined and sampled for diagnostic purposes. The specimens are normally held for four weeks and then disposed. A request for return of the specimens must be made at the time of surgery or within four weeks.



Patient Name: Fatma Mahmoud Mohamed Mohamed Saad	Date : 22/09/2024
Exam : MAMMOGRAPHY BOTH	Age : 54 Year(s)
Medical ID: 3225/2024	Pat. ID: 79364

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast cancer

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows upper quadrants spiculated irregular mass lesion, of high density with microcalcifications measuring 12 x 9 cm and reaching nipple as well diffuse skin thickening.
- Complementary ultrasound revealed left breast irregular spiculated hypoechoic mass lesions, with vascularity detected within on color Doppler, the largest seen at 2 to 3 o'clock position measuring 2 x 3.2 cm and posterior satellite lesion abutting pectoral muscle measures 0.6 x 1 cm. suspected anterior ductal extension reaching nipple.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Pathological left axillary level I, II & III lymph nodes are seen with muffled hila, the largest appear amalgamated measuring 4.8 x 2.2 cm
- Nonspecific right axillary LN.

Opinion:

- Left breast UOQ locally advanced multicentric malignant mass lesions (BIRADS 6)
- Pathological amalgamated left axillary LNs.

Yours truly

Signature Dr. Wafaa Ali

22/09/2024 4:38 PM



Patient Name: Fatma Mahmoud Mohamed Mohamed Saad	Date : 17/10/2024
Exam : TRIPHASIC ABDOMEN & PELVIS	Age : 54 Year(s)
Medical ID: 3225/2024	Pat. ID: 79364

**MULTI-SLICE CT SCAN OF THE ABDOMEN AFTER CONTRAST
ADMINISTRATION WITH TRIPHASIC LIVER STUDY: -**

Findings:

Known case of left breast cancer.

- Enlarged liver showing homogenous CT texture and regular outline. Small about 1.2 x 1.4 cm focal lesion is seen at segment II of left hepatic lobe being not clearly visualized in arterial phase and become hypodense to liver parenchyma in portal and delayed phases.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- No abdominal lymph node enlargement seen.
- No ascites nor abdominal collections noted.
- Clear scanned lung bases.

Opinion:

Hepatomegaly showing left hepatic lobe focal lesion as described, worrisome for metastatic deposit

Dr. Shaimaa Khaled, MSc.

Dr. Ibrahim Abo El Magd, M.D.



Exam Information:

Patient Name: Fatma Mahmud	Date of birth: 22/06/1970
Arabic Name: فاطمة محمود	Patient Gender: F
Patient RIS ID: 366709	Exam Date: 05/03/2025
Patient HIS ID: 10110225004100	
Written by: Dr. Hoda Fathy	Approved by: Dr. Hoda Fathy

^{99m}Tc MDP Bone Scan Report (05/03/2025)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP
Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen ali over the surveyed skeleton.

Yasmeen El Araby

Much Obliges

Mohamed Abdelawab, MDSc



DNIS
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Medical No. : 2371

Patient's Name: Fatima Mahmoud Mohammed

Date: 2026-09-19

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Left breast carcinoma on 'NACT', follow ups study.

Findings:

Right breast.

- Scattered densities of glandular tissue . (ACR B)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- Evidence of benign macrocalcification.
- **Ultrasound** demonstrated no solid or cystic lesions.

• Normal skin/nipple/areola complex.

• Non-specific axillary LNs.

Left breast.

• Scattered densities of glandular tissue . (ACR B)

• Thickened left breast skin with underlying large infiltrative central breast mass with microcalcifications within ,the whole affection is over an area of 7.8 x 6.5 cm .

• Ultrasound demonstrated multiple irregular shaped masses are seen in between 11-2 o'clock p[osition ,the largest mass is seen t 1 o'clock ,zone 3 measures (3.9 x 3 cm).

• Infiltearted nipple .

• Multiple (more than 3 In number) pathological levels 1,2 and 3 axillary LNs ,the largest measures (3.7 x 1.8 cm).

Conclusion:-

- *Right breast (BIRADS 1): Normal .*
- *Left breast (BIRADS 6): progressive course as regard the previously mentioned invasive masses .*
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Prof.MONA ZAKY